

Breastfeeding: Definition, Promotion, Education and Support Standards

Policy: Local agencies shall provide breastfeeding promotion, education and support to participants and designate a Local Agency Breastfeeding Coordinator.

Regulations: CFR 246.11 c(2),(5),(7)(i)-(iv), CFR 246.2, CFR 246.10 e, USDA FNS WIC Breastfeeding Policy and Guidance (July 2016), Colorado WIC Policy

Definitions:

Breastfeeding Recommendations: Human milk is the optimal food for infant growth and development. The American Academy of Pediatrics (AAP) recommends exclusively breastfeeding for the first 6 months after birth, and supports continued breastfeeding for 2 years or as long as desired.

Exclusive Breastfeeding: An infant does not receive any additional foods or fluids other than breast milk unless medically recommended.

Breastfeeding Woman: Refers to women up to one year postpartum who are providing their breast milk to their own infant(s), including:

- Women who are providing any amount of breast milk to their infant(s) via breastfeeding (exclusively or partially).
- Women who are providing any amount of breast milk to their infant(s) by expressing the breast milk by hand or pump and feeding the expressed breast milk to the infant.
- Women who are providing any amount of breast milk to infant(s) to whom they did not give birth **and for whom she has legal responsibility**. Examples include foster mothers, adoptive mothers, and female partners in the same household. A non-birth breastfeeding woman may receive WIC services if she meets eligibility criteria and the infant receiving her breast milk is a WIC participant.

Breastfeeding Descriptions Definitions*:

- Exclusively Breastfeeding ("Excl BF"): Infant fed only human milk, vitamins, minerals, and/or medications. Infant receives no food package from WIC.
- Primarily Exclusive/No Food Pkg ("Prim Excl/No F Pkg"): Infant fed something other than human milk, vitamins, minerals, and/or medications on rare occasions, or has received a limited-time (not routine) feeding of infant formula, human milk fortifier, cow's milk, juice, sugar water, rehydration solution, baby food, or anything else. Includes infants whose exclusive breastfeeding is interrupted because of special circumstances, such as acute illness, hospitalization, or caregiver misinformation. Infant receives no food package (no infant formula) from WIC.
- Primarily Exclusive/Complimentary Foods ("Prim Excl/Comp"): Infant fed any complimentary food in addition to only being fed human milk, vitamins, minerals, and/or medications. These complimentary foods are provided on a routine or ongoing basis, regardless of the amount. Infant receives age-appropriate food packages with no WIC infant formula.
- Partially Breastfeeding In Range ("Part BF - in range"): Infant fed human milk and something other than human milk, such as complementary foods, infant formula (non-WIC supplied or from WIC up to the maximum provided for a mostly breastfeeding infant - described in Table 1 of 7 CFR 246.10).
- Partially Breastfeeding Out of Range ("Part BF Woman - out of range"): Infant fed human milk and receives more than the maximum amount of infant formula from WIC provided for a mostly breastfeeding infant, but less than the amount provided for a non-breastfeeding infant.
- No Longer Breastfeeding ("No Longer BF"): Infant was breastfeeding at some point in time, but has now discontinued.
- Never Breastfed ("Never BF"): Infant was never breastfed. Should only be used if breastfeed was never initiated.
- Breastfeeding Child ("BF Child"): Child greater than or equal to 12 months of age continuing to receive human milk.

*Note: does not include clients providing breast milk to infants only by donating their breast milk to a milk bank or other source.

Procedure/Additional Guidance:

Promotion of Breastfeeding

Colorado WIC will promote and support breastfeeding.

- Each local agency program will designate a staff person to serve as the Breastfeeding Coordinator to provide leadership to the local WIC agency on breastfeeding promotion, education and support matters.
- All agency staff exhibits a positive attitude toward breastfeeding and, depending on their position, be expected to encourage, educate on, promote and support breastfeeding and make referrals for breastfeeding services beyond WIC. See *Breastfeeding Roles in Colorado WIC Clinic* in the *Breastfeeding: Scope of Practice* policy.

Staff Training

Federal regulations require local agency staff to have a basic knowledge of breastfeeding and understand their unique role in supporting breastfeeding as the standard method of infant feeding. All staff have a valuable role in supporting breastfeeding women in WIC.

- The local agency Breastfeeding Coordinator helps train staff in the provision of breastfeeding support services and serves as a mentor for new and existing staff.
- All new staff must complete at a minimum the online WIC Lactation Specialist (WLS) training series Modules 1-3, as well as additional task-appropriate breastfeeding training as determined by the local agency Director and/or Breastfeeding Coordinator.
- State office will provide in-service training and technical assistance on the promotion and management of breastfeeding to staff at local agencies.

Clinic Environment

All local agencies must develop policies and create a clinic environment that endorses breastfeeding as the preferred method of infant feeding. A positive breastfeeding clinic environment demonstrates to mothers the importance WIC places on breastfeeding and enhances staff efforts in promoting breastfeeding.

- Encourage staff to communicate a positive attitude toward breastfeeding.
- All printed materials (e.g., handouts, pamphlets, posters), audiovisual materials and social media messages used will use positive breastfeeding messages and images and be free of infant formula product names or manufacturer information or logos.
- Ensure infant formula, bottles, feeding supplies and related materials are out of the sight of participants and the public.
- Office supplies (e.g., cups, calendars, pens, note pads, etc.) must be free of infant formula product and manufacturer/sponsor names and information.
- Refuse free gifts and samples from infant formula representatives for use by local agency staff.
- Encourage and support local agency staff to breastfeed their children.
- Support breastfeeding in clinics and provide private space (other than a toilet stall) for staff and participants to breastfeed and/or express breast milk.
- Develop a mechanism to incorporate positive peer influence into the clinic. This could include a breastfeeding affirmation bulletin board where pictures and statements from breastfeeding moms and infants are displayed (from staff and participants), and/or classes or support groups where breastfeeding women and/or peer counselors talk to pregnant women.

Education and Support

Breastfeeding education and support are provided throughout the prenatal and postpartum period, particularly at critical times when the mother is most likely to need assistance. All women should be educated on the benefits of exclusive breastfeeding.

Prenatal Education

Prenatal education and breastfeeding intention prior to delivery is one of the best predictors of a woman's breastfeeding success. Every opportunity should be taken during a woman's pregnancy to educate on the importance of breastfeeding, address breastfeeding concerns and questions, and provide encouragement to initiate and continue breastfeeding as long as possible.

- The local agency will use prenatal breastfeeding education protocols which:
 - ✓ Integrate breastfeeding promotion into the continuum of prenatal nutrition education.
 - ✓ Include an assessment of participant knowledge, concerns, attitudes and support networks related to breastfeeding.

- ✓ Provide initial and follow-up breastfeeding education sessions to each participant and include breastfeeding education at every prenatal participant visit and interaction.
- ✓ Are comprehensive, up-to-date, and culturally appropriate.
- ✓ Include information on the benefits of exclusive breastfeeding and practical strategies to combat potential challenges.
- ✓ Include information to prepare mothers to communicate effectively with hospital staff of her decision to breastfeed.
- ✓ Include information to prepare mothers to communicate effectively with her employer/school as she prepares for maternity leave.
- Staff must invite all pregnant women to a breastfeeding/infant feeding class or provide comparable education one-on-one with participants.
- Staff will follow local agency systems in place to refer pregnant women to the Breastfeeding Peer Counseling program.

Postpartum Education

The first few weeks postpartum are critical time periods in helping mothers build a milk supply and confidence. Research shows mothers with early and continued breastfeeding support are the most likely to succeed and breastfeed for longer durations.

- Staff will make efforts to schedule mothers who intend to breastfeed for subsequent certification as soon after delivery as possible in order to provide timely breastfeeding support.
- Inform participants that WIC helps mothers establish a successful breastfeeding relationship with their infant(s) and provides support to build their milk supply.
- All postpartum contacts provide positive reinforcement for the continuation of breastfeeding.
- Staff will document a mother's plan for breastfeeding in her nutrition education record and provide education and support to help her achieve her breastfeeding goals.
- Staff will develop a plan to assure that women have access to locally available support programs/services early in the postpartum period (when infant is 0-2 weeks of age).
- Local agencies will invite women to a WIC support group or refer women to a community breastfeeding support group.
- Staff will provide and/or identify education and support services for breastfeeding women returning to work or school.
- Local agencies collaborate with other programs/groups that provide breastfeeding education/support.
- Staff will follow local agency systems in place to refer new mothers and breastfeeding women to the Breastfeeding Peer Counseling program.

Formula

When a breastfeeding woman requests formula for her breastfed infant, the first priority is to help the woman achieve her breastfeeding goals. Staff must:

- Perform a nutrition assessment and use participant centered approaches to understand each mother's individual request for formula supplementation and determine the appropriateness of supplementation.
- Ask open-ended questions to understand the reasons behind the infant formula request and assess a mother's understanding of the potential impact of formula on milk supply and breastfeeding.
- Continue to affirm, encourage and support any amount of breastfeeding.
- Inform the mother that her food package will change based upon how much she is breastfeeding.
- Document a mother's plan for breastfeeding, formula use and education provided in her nutrition education record.
- When formula is issued to a breastfed infant:
 - Provide only the minimum amount to meet, but not exceed the infant's nutritional needs. Remember that WIC is a supplemental program and mothers will have greater breastfeeding success if they continue to breastfeed and/or express their milk regularly.
 - Provide powdered formula as a supplement because it can be prepared in as small a quantity as needed.
 - Determine the amount of supplement necessary that meets but does not exceed the infant's nutritional needs and assign the appropriate food package. A breastfeeding infant who receives any formula from WIC is no longer in the fully breastfeeding category and needs to be adjusted

appropriately.

- Adjust the mother's food package appropriately based upon her level of breastfeeding and amount of infant formula issued. A woman over six months postpartum and determined to be in the "some breastfeeding" category will not receive a food package, however can still receive all other WIC services.
- Document appropriately in the Compass computer system.
- Provide regular follow up with the mother at least monthly to continue to support breastfeeding success as able.

Banked Human Milk

Banked human milk is not a WIC-eligible supplemental food (as defined by 7 CFR 246.10) and WIC federal funds cannot be used for the purposes of collecting, storing, purchasing, or disseminating banked human milk. WIC staff may inform parents about banked human milk, refer clients to human milk banks, and inform parents about donating their milk. Additionally, WIC clinics can serve as a human milk depot in partnership with another organization/human milk bank. Prior to serving as a milk depot, a Memorandum of Understanding (MOU) is required between the WIC agency and human milk bank/organization to clearly define the roles and responsibilities of each organization and explicitly state that banked human milk is not a WIC benefit, nor is the milk depot sponsored by WIC.